

AN ACADEMIC RECOVERY WORKBOOK

STAT SUCCESS

When your grade can't wait.

Failure is what happened.

It is not who you are.

Triage, stabilize, treat, and monitor your own course
the way a clinician would. Eight weeks, self-paced.

DR. SHARILYN RENNIE

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STAT Success Workbook: When Your Grade Can't Wait

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This workbook is an educational tool, not a substitute for academic advising, mental health support, or institutional accommodations. If you are in crisis, reach out to your campus counseling center or a licensed clinician.

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A LETTER FROM THE AUTHOR

You have not failed.

Something happened. That is not the same thing.

Failing a course is not who you are. It is what happened. Those are not the same thing, and almost every student who picks up a workbook like this one is conflating them. If you have been telling yourself that you are a failure, or that you are not smart enough, or that this is just who you are, stop. That is the shame talking, and the shame is wrong about you.

Here is what is true. Your grade is data, not identity. A failing grade is fixable. Even the worst case, you fail the class, is a recoverable loss. The only outcome that is not recoverable is giving up. As long as you are still in the work, the door is open.

I built STAT Success for the moment you are in. Most study programs are built for students who are already stable, who have time, and who want to improve from a B to an A. This is not that. This is a clinical method for academic emergencies. You will triage your own situation the way a clinician triages a patient. You will diagnose what is actually going wrong (rarely what you think). You will stabilize first, treat next, and monitor what is working week by week.

The work in here is honest. It will ask you to look at numbers you have been avoiding. It will ask you to name habits you do not want to name. Do the work anyway. Recovery is analyzing, planning, restructuring. None of those steps requires you to be exceptional. They require you to be present. You are reading this, which means you have not given up. That is enough to start.

AVAILABLE ONLINE Online companion tool hub

Most worksheets in this book have a matching interactive tool online. Same content, auto-scored. Use the hub for the math; use the book to keep your work.

drsrennie-stack.github.io/STAT/hub.html

Yours in recovery,

Dr. Sharilyn Rennie

Founder, MedMasters Collaborative

BEFORE THE WORK BEGINS

A note on what failure is, and what it isn't.

Failing a course is not a permanent state. It is a temporary one, made permanent only if you decide to leave it permanent. Recovery is available almost every time. The only time it is not available is when the student stops doing the work.

Failing a course does not mean you are a failure. Failure is something that happened. It is not who you are. Confusing the event for the identity is the single most common thing that turns a recoverable situation into a quit. Notice when you do this. Name it. Then return to the work.

Even the worst case is recoverable. Suppose every effort in this workbook fails, and you fail the course. You will have lost a semester. You will not have lost your future. A failed class is a delay, not an ending. A repeated class often produces a stronger student than a passed one. The only outcome that does not recover is the one where you decide, before the data is in, that the recovery is not possible. Do not decide that. The data is not in yet.

Recovery is work. It is analyzing what went wrong, planning what to do differently, and restructuring how you operate. None of those steps requires you to be smart enough or talented enough or anything enough. They require you to be present, honest, and willing to try one thing this week that you did not try last week. That is the entire skill.

Failure is what happened. It is not who you are. And it is fixable, with the right tools and the right method, by people who decide to stay in the work.

You have not given up. You are reading this. That is enough to start.

BEFORE YOU BEGIN

How to use this workbook.

This workbook follows the same arc as the STAT Success eight-week cohort program: an honest assessment of where you are, an eight-week guided cycle, and a structured plan for what comes after. You will do it on your own, at your own pace.

THIS IS A RECOVERY WORKBOOK. NOT A PREVENTION WORKBOOK.

How you approach a course when you are not doing well is very different from how you approach a course you are starting fresh, or one you are doing well in. Recovery is triage: narrow focus, stop the bleed, accept partial coverage in low-yield areas. Prevention is comprehensive: broad coverage, deliberate practice, room for depth. This book teaches the first. Part 4 hands you back to the second.

Three things to know up front

- **Start with Part 1.**
The diagnostic, the grade audit, and the decision flowchart in Part 1 are not optional. They tell you which weeks to weight most heavily and what to expect.
- **Write in this book.**
The workbook is fillable. Use a pencil if you want to revise. Honesty matters more than neatness. Falsified self-assessment is the most reliable predictor of failure in recovery.
- **Pace yourself one week at a time.**
You can read the whole book in an afternoon. You cannot do the work in an afternoon. Each weekly section is designed to take 60 to 90 minutes plus your normal study time. Do not skip ahead.

Workbook map

- | | |
|---------------|--|
| PART 1 | Triage. Diagnose your acuity and your failure modes. Build your decision plan. |
| PART 2 | The CODE method. The weekly cycle: Current state, Observe, Decide, Evaluate. |
| PART 3 | Eight weekly chapters. One case per week, your own audit, your own plan. |
| PART 4 | Discharge. What survives the program, what fades, and how to come back. |

WORKBOOK CONTENTS

Contents.

A note on what failure is, and what it isn't

PART 1 Triage

The three acuities
Grade audit worksheet
Failure mode diagnostic (24 questions)
Scoring & interpretation (write-in)
The decision flowchart

PART 2 The CODE method

Current state, Observe, Decide, Evaluate
The four clinical phases

PART 3 The eight weeks

Week 1 · Stabilization (+ optional partner)
Week 2 · Stop the bleeding · Morgan
Week 3 · Foundation repair · Jordan
Week 4 · The midterm hinge · Alex
Week 5 · Point recovery · Riley
Week 6 · Consolidation & integration · Sam
Week 7 · Final exam runway · Casey
Week 8 · Definitive care & discharge · Taylor

PART 4 After STAT

Discharge planning worksheet
Recovery vs. prevention
Warning signs & re-entry
Quick reference card

PART 1

Triage.

When your grade can't wait.

PART 1 · YOUR SITUATION HAS AN ACUITY

The three acuities.

Different acuities call for different strategies. The first job is to diagnose your own.

LEVEL 1

Stable

WHO YOU ARE

Grade in the B range. More than four weeks until your next exam. Confidence across most topics.

STRATEGY

Long-term mastery, deep learning, deliberate practice across topics.

WHAT TO DO WITH THIS WORKBOOK

You may not need this workbook yet. Bookmark it for later in the semester.

LEVEL 2

Urgent

WHO YOU ARE

Grade in the C range. Two to four weeks until your next exam. Low confidence in specific topics.

STRATEGY

Targeted intervention on diagnosed weak areas. Protocol-driven practice. Daily monitoring.

WHAT TO DO WITH THIS WORKBOOK

This workbook is built for you. Move through Parts 1–3 in order.

LEVEL 3

Critical

WHO YOU ARE

Grade at or below passing. Less than two weeks until your next exam. Widespread uncertainty.

STRATEGY

Stabilize first. Treat highest-yield topics only. Accept partial coverage. Triage hard.

WHAT TO DO WITH THIS WORKBOOK

Start with Part 1, then skip to Week 7 (final exam runway). Return to weeks 2–6 after you stabilize.

Circle yours, then turn the page.

PART 1 · WHERE YOU ACTUALLY ARE

Grade audit.

Fifteen minutes. Open your syllabus and your gradebook. Be honest, not aspirational.

Your course

Course name & number

Instructor

Current grade (%)

What grade do you need to pass?

What grade is your goal?

Weeks into the semester

Weeks remaining

Next major assessment & date

What is still left to earn?

List every remaining graded item. Be specific. This is the math of your recovery.

ASSIGNMENT / EXAM	WEIGHT (%)	DUE / DATE	TARGET SCORE

REALITY CHECK

Add the weights in column 2. If they total much less than 100% minus what is already locked in your current grade, your math is off. Recheck before continuing.

PART 1 · WHY YOU ARE FAILING, MORE SPECIFICALLY

Failure mode diagnostic.

24 statements. Four dimensions. About 8 to 10 minutes.

Most students who are failing a course are not failing for one reason. They are failing for a specific combination of reasons, and the recovery path depends on which reasons dominate. This diagnostic looks at four dimensions of academic failure and produces a profile specific to you.

How to answer

Rate each statement on a 1 to 5 scale, where 1 means "not true of me right now" and 5 means "deeply true of me right now." Be truthful, not aspirational. Rate how true the statement is right now, in your current course, not how you wish it was or how you plan to change.

THE SCALE

1 Not true of me right now. **2** Slightly true. **3** Moderately true. About half. **4** Mostly true. **5** Deeply true right now.

THE PROFILE IS ONLY USEFUL IF THE INPUT IS HONEST.

Students who falsify their self-assessment recover slower than students who tell the truth, because falsified data hides the pattern that needs fixing. Rate where you are, not where you want to be.

AVAILABLE ONLINE Failure Mode Diagnostic (online)

Take the 24 questions on the hub if you would rather have it auto-scored. Then bring your four subtotals back to the book and write them in.

drsrennie-stack.github.io/STAT/hub.html

PART 1 · DIMENSION · TIME

Time scarcity.

	1	2	3	4	5
	not true	deeply true			
1. I run out of time to study before every major assessment.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
2. Work, family, or health obligations take priority in ways I cannot control.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
3. I know I need to study more, but I cannot find the hours in my schedule.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
4. I waste time on non-essential things when I could be studying.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
5. I start assignments late and run out of time to do them well.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
6. My week rarely goes how I plan it to go.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5

YOUR TIME SUBTOTAL

Add the numbers you circled above (min 6, max 30).

PART 1 · DIMENSION · METHOD

Study method failure.

	1	2	3	4	5
not true	deeply true				
1. When I study, I mostly re-read notes or textbook sections.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
2. I highlight key terms and passages as I read.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
3. I make or use flashcards for vocabulary and definitions.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
4. I feel like I am studying, but the material does not stick.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
5. I wait until close to an exam to seriously study.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5
6. I spend hours on one topic but cannot explain it afterward.	☐ 1	☐ 2	☐ 3	☐ 4	☐ 5

YOUR METHOD SUBTOTAL

Add the numbers you circled above (min 6, max 30).

PART 1 · DIMENSION · CONTENT

Content gaps.

	1	2	3	4	5
	not true		deeply true		
1. I feel like I am missing foundation concepts from earlier units.	☐	☐	☐	☐	☐
2. There are chapters or topics I skimmed that I am now paying for.	☐	☐	☐	☐	☐
3. Even when I study, I do not understand the "why" of what I am learning.	☐	☐	☐	☐	☐
4. Terms blur together and I cannot keep them straight.	☐	☐	☐	☐	☐
5. In lectures or exams, other students seem to understand something I do not.	☐	☐	☐	☐	☐
6. I struggle to connect concepts across body systems or units.	☐	☐	☐	☐	☐

YOUR CONTENT SUBTOTAL

Add the numbers you circled above (min 6, max 30).

PART 1 · DIMENSION · AFFECT

Affective overload.

These questions ask about shame, panic, and identity. Naming a pattern is not the same as being the pattern. Answer honestly. The pattern is fixable.

	1	2	3	4	5
	not true	deeply true			
1. I feel panic or dread when I think about studying.	☐	☐	☐	☐	☐
2. I avoid studying even when I have time because it feels overwhelming.	☐	☐	☐	☐	☐
3. When I get a bad grade, I struggle to open the gradebook for days.	☐	☐	☐	☐	☐
4. I believe I am not smart enough for this material.	☐	☐	☐	☐	☐
5. If I cannot study perfectly, I often do not study at all.	☐	☐	☐	☐	☐
6. I feel ashamed about how far behind I am, and that shame makes it harder to catch up.	☐	☐	☐	☐	☐

YOUR AFFECT SUBTOTAL

Add the numbers you circled above (min 6, max 30).

PART 1 · YOUR PROFILE · SCORES

Write your scores in.

Pencil works best. You will come back to this page.

Transfer your four subtotals into the boxes. The highest is your primary failure mode. The next is your secondary. The lower two are not currently driving your situation. Write your scores in this book so you can return to them in Week 6, when you re-read your own diagnosis.

TIME	METHOD	CONTENT	AFFECT
(write 6 – 30)	(write 6 – 30)	(write 6 – 30)	(write 6 – 30)

Your profile

Primary failure mode:

The highest score above. This is what to address first.

Secondary failure mode:

The second-highest. Often masked by the primary.

Total score (sum of 4):

Add all four subtotals. Helps gauge overall acuity.

Total score acuity bands

24 – 48 Stable.	Not in crisis. Address your highest subscale only. Skim Parts 2–3.
49 – 72 Urgent.	Targeted intervention. Move through all 8 weeks in order. Primary failure mode is your start.
73 – 96 Critical.	Multiple modes elevated. Stabilize first (Weeks 1–2). Triage time. Talk to your instructor.
97 – 120 Crisis.	Every dimension elevated. Reach out to campus advising or counseling this week. Then return.

PART 1 · YOUR PROFILE · INTERPRETATION

Write what this means for you.

A score is a number. The interpretation is the work.

Use this page to put your scores into your own words. You will return to this page in Week 6, when you compare your Week 1 diagnosis to five weeks of additional data. The clearer you write it now, the more useful it is then.

My primary failure mode is:

Write the name (Time, Method, Content, or Affect). Then write what this looks like in your real life, this week. Specific examples beat abstractions.

My secondary failure mode is:

Same. Be specific. Secondary modes often hide behind the primary.

My acuity band is:

Stable, Urgent, Critical, or Crisis. Circle one. Then write: what does this acuity mean for the next 8 weeks of my life?

What I notice that surprised me:

A statement you scored higher (or lower) on than expected. Why?

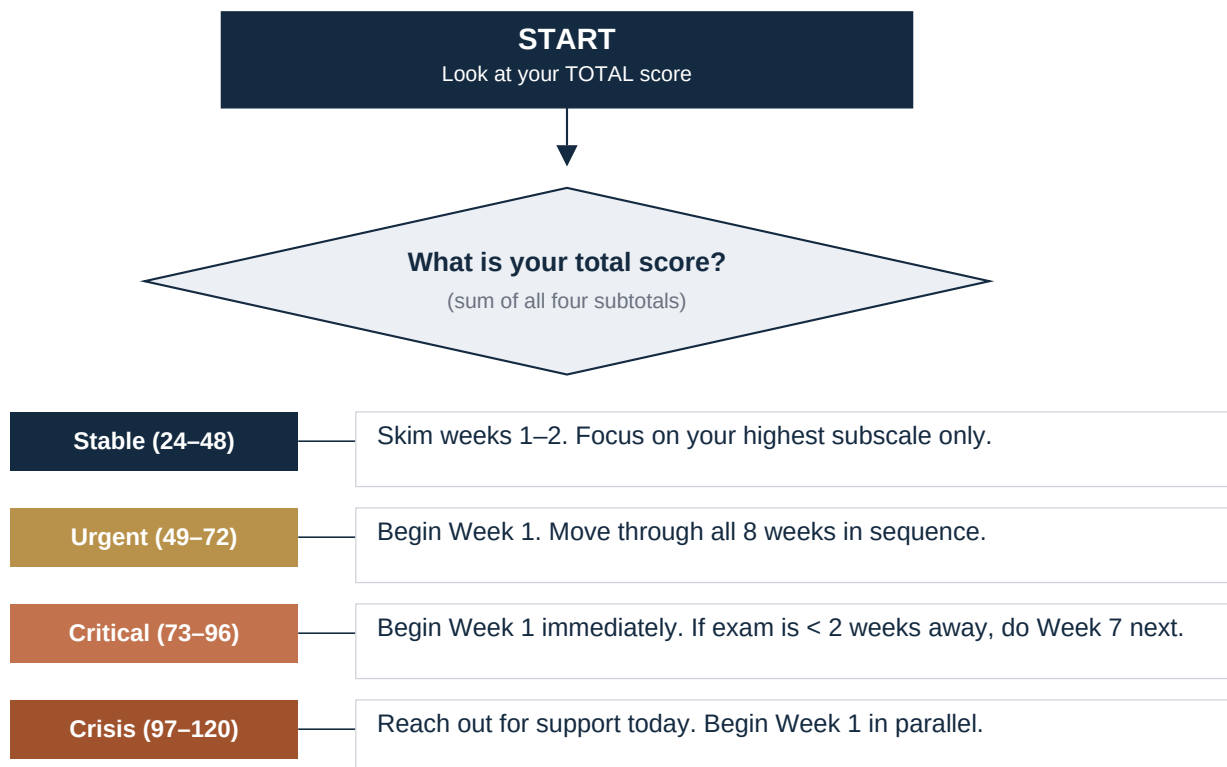
What I plan to do first, based on this diagnosis:

One sentence. Specific, not aspirational.

PART 1 · WHAT TO DO NEXT

The decision flowchart.

Use your primary failure mode and total score to find your first action.



NEXT · YOUR PRIMARY FAILURE MODE TELLS YOU WHICH WEEKS WEIGH MOST

TIME	Time scarcity Weight Week 1 (audit) and Week 2 (stop-the-bleed). Build your study calendar before Week 3.
METHOD	Study method Week 2 is your week. Slow down here. Do not move on until your method has actually changed.
CONTENT	Content gaps Week 3 is your week. Foundation repair is the work. Selective, not comprehensive.
AFFECT	Affective load Week 1 and Week 5 are your weeks. Do not skip the leading-indicator work in Week 5.

PART 2

The CODE method.

When your grade can't wait.

PART 2 · THE WEEKLY CYCLE

C – O – D – E.

Every week of STAT Success runs the same four-step loop.



Current state

A weekly self-audit using a SOAP-note structure: Subjective (how it felt), Objective (what the data says), Assessment (what is actually going on), Plan (the implication).



Observe

A guided case study of another student in your situation. You will read it, analyze it, and borrow the patterns. Other people's mistakes are easier to see than your own.



Decide

Three specific commitments for the week ahead, plus one concrete action you will take in the next 24 hours. Vague plans fail. Specific plans get done.



Evaluate

A short honest review of last week's commitments. Did you do them? Did they work? Evaluate is not for performance assessment. It is for data.

THE LOOP NEVER STOPS

Each week's Evaluate becomes next week's starting point. By Week 4 the loop is honest. By Week 6 it is automatic. By Week 8 it is yours.

PART 2 · WHERE THE EIGHT WEEKS LIVE

Four clinical phases.

Triage. Stabilize. Treat. Monitor. Applied to your own learning.

i

Triage

WEEKS 1

Diagnose acuity, identify the two or three topics that are bleeding points, rank by urgency. Output: an acuity score and ranked priorities.

ii

Stabilize

WEEKS 2–3

Immediate interventions on the highest-priority topics. Stop methods that are not working. Repair the foundation gaps that block current material. Output: a stabilization plan.

iii

Treat

WEEKS 4–6

Deep intervention on the diagnosed weak topics. Read midterm data honestly. Sustain what is working. Revisit your diagnosis if results do not match effort. Output: measurable mastery.

iv

Monitor

WEEKS 7–8

Final exam triage. Discharge planning. Build the structure that replaces this one. Output: sustained improvement, and a plan that survives without the workbook.

PART 3

The eight weeks.

When your grade can't wait.

WEEK 1 · PHASE: TRIAGE

Stabilization.

Before you can change anything, you have to see it clearly.

Week 1 is where the program starts. Stabilize, see your situation clearly, and make your first set of specific commitments for the week ahead. In clinical critical care, stabilization comes first. You do not treat the long-term problem until the patient is stable enough to receive treatment. Week 1 is the same. You will likely be working alone; that is fine and that is the design. If you have one willing person who can check in with you weekly, see the optional partner sidebar at the bottom of this page.

Your focus this week

Do not try to catch up on the entire course this week. Do not try to rebuild all your study habits at once. The goal of Week 1 is stabilization, not transformation. Go to class. Turn in what is due. Complete your grade audit and your decision plan. That is the entire assignment.

What you will do

- ☐ Complete your grade audit (Part 1, if not already done).
- ☐ Take the failure mode diagnostic (Part 1).
- ☐ Write your interpretation in the book so you can return to it in Week 6.
- ☐ Read your "Where to start" results in the decision flowchart.
- ☐ Write your Week 1 decision plan (this section).
- ☐ Optional: name one accountability partner and tell them your three commitments.
- ☐

OPTIONAL Working with someone? One protocol.

Most readers will do this workbook alone. That is the default and the design. If you have one person willing to do this alongside you, a sibling, friend, classmate, study partner, this is the minimum structure: one weekly 30-minute check-in. Share your three commitments. Be honest about whether you did them. That is it. You do not need to work the same course or the same material to hold each other to honest weekly reporting.

WEEK 1 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

Your Plan (P) becomes your Decision Plan on the next page.

WEEK 1 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

1

2

3

One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

WEEK 2 · PHASE: STABILIZE

Stop the bleeding.

Recognition comes first. Change comes second.

Week 2 is where the rhythm starts. The full CODE cycle runs for the first time: an Evaluate check on Week 1, your Current state audit, your work on Morgan's case, and your own decision plan coming out of what Morgan's case surfaces. In critical care, the first priority with an unstable patient is stopping what is actively making things worse.

Your focus this week

Pick one method you currently use that Morgan's case helped you see differently. Stop doing it, or do it differently. That is enough. Small changes that actually happen are worth more than large changes that do not.

What you will do

- ☐ Read Morgan's case (next page).
- ☐ Complete your Week 2 Current state audit.
- ☐ Answer the three case questions in writing.
- ☐ Write your Week 2 decision plan with one specific method change.
- ☐

WEEK 2 · CASE STUDY

Morgan's case.

The student who studies harder than anyone they know, and is failing anyway.

CASE SNAPSHOT

Course	Anatomy & Physiology 2
Week of semester	4 of 16
Current grade	58%
Exams completed	2 of 5 (56%, 38%)
Lab practicals	2 of 5 (38%, 58%)
Hours studied / week	15 – 18 (self-reported)
Office hours visits	0

Morgan in their own words

I do not understand what is happening. I study harder than anyone I know. I put in at least 15 hours a week. My notes are beautiful. I color-code them by system. I reread the chapters before lectures and again before exams. But when I sit down for the exam, my mind goes blank. I leave the exam feeling like I did not know half of what was asked. I am starting to wonder if I am just not smart enough for nursing.

What is actually happening

- Reads each chapter with a highlighter before lecture.
- Takes handwritten color-coded notes during lecture.
- Rewrites notes after class.
- Re-reads textbook sections in the days before each exam.
- Uses Quizlet for vocabulary almost daily.
- Watches recommended YouTube videos, sometimes twice.
- Has not attended a single office-hours session.

WHAT THIS CASE IS TEACHING

Morgan is doing passive review. Every activity on Morgan's list feels like studying and produces almost no retention. The hours are real. The work is not.

WEEK 2 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

E · Evaluate last week

Did you do what you said you would? Be specific, not performative.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

Your Plan (P) becomes your Decision Plan on the next page.

WEEK 2 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

1

2

3

One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

WEEK 3 · PHASE: STABILIZE

Foundation repair, selective.

You cannot close every gap this week. You are not supposed to.

Week 3 turns from method to content. You have a new study approach from Week 2. Now the question is what you actually understand, what you do not, and which of the gaps are blocking you right now. Foundation repair is selective, not comprehensive. You fix what is actively preventing you from getting better.

Your focus this week

Identify one to two specific foundation concepts that are blocking current material. For each one, find a source (a YouTube video, a textbook chapter, an office-hours visit, a tutor) and close the gap. Specificity is what makes foundation repair possible. "I need to review A&P 1" is not a gap. "I do not understand how the sodium-potassium pump establishes resting membrane potential" is a gap.

What you will do

- ☐ Read Jordan's case.
- ☐ Complete your Week 3 Current state audit.
- ☐ List the foundation concepts you suspect are missing.
- ☐ Pick one or two and plan how to close them this week.
- ☐

AVAILABLE ONLINE Foundation Gap Identifier (online)

Walks you through three prompts about your current coursework and surfaces which earlier concepts are blocking new material. Recommended this week.

drsrennie-stack.github.io/STAT/hub.html

WEEK 3 · CASE STUDY

Jordan's case.

The student who fixed their method, and is still failing.

CASE SNAPSHOT

Course	Anatomy & Physiology 2
A&P 1 grade	C-minus
Current grade	62%
Lecture exams	3 of 5 (65%, 60%, 61%)
Lab practicals	2 of 5 (58%, 55%)
Hours studied / week	18 – 22
Office hours visits	1

Jordan in their own words

I did the work. I stopped rereading. I got rid of the color-coded notes. I bought a practice question bank and I actually use it. I self-test without notes in front of me. And yet the exam last week was a 61. When I try to explain the resting membrane potential out loud, I cannot do it. I do not understand what a concentration gradient really is. It is like the professor is building a house and everyone else has the foundation already, and I am standing on sand.

What is actually happening

- Switched from passive re-reading to active recall.
- Uses a practice question bank almost daily.
- Self-tests without notes.
- Attended one office-hours session.
- Studies more hours, more efficiently.
- Cannot explain core concepts out loud when asked.

WHAT THIS CASE IS TEACHING

Jordan no longer has a method problem. Jordan has a foundation problem. Method improvement cannot fix missing prerequisites. The work of Week 3 is to find them and close the most critical ones, selectively.

WEEK 3 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

E · Evaluate last week

Did you do what you said you would? Be specific, not performative.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

Your Plan (P) becomes your Decision Plan on the next page.

WEEK 3 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

1

2

3

One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

WEEK 4 · PHASE: TREAT

The midterm hinge.

Midterm results are data, not verdict.

Week 4 is the midpoint. Most students will have just taken a midterm or major exam, or will take one this week. This is the first time the work of recovery meets the real data. The job this week is to read the data honestly and decide what stays, what changes, and what needs to be added.

Your focus this week

Your decision plan this week has three parts. PROTECT: what is working that you will keep doing regardless of whether numbers have moved yet. PIVOT: what is not working and needs to change. ADD: what has not been part of your plan that the data showed you needed.

What you will do

- ☐ Read Alex's case.
- ☐ Complete your Week 4 Current state audit, with midterm data honestly reported.
- ☐ Use the Exam Wrapper on the next page if you have a midterm back.
- ☐ Write your Week 4 plan: Protect / Pivot / Add.
- ☐ Complete the mid-program reflection.

AVAILABLE ONLINE Exam Wrapper + Mid-Program Reflection (online)

Two tools for Week 4: a structured reflection on the midterm and a halfway-point review of the whole program so far. Both worth your time this week.

drsrennie-stack.github.io/STAT/hub.html

WEEK 4 · CASE STUDY

Alex's case.

The midterm came back. The results are mixed. Now what?

CASE SNAPSHOT

Course	Anatomy & Physiology 2
Grade four weeks ago	52%
Current grade	62%
Midterm lecture score	68% (up from 51%)
Midterm lab practical	54% (essentially flat)
Hours studied / week	20 – 24

Alex in their own words

My lecture score jumped seventeen points. I should feel good. But the lab practical barely moved. The method change worked for lecture. It did not transfer to lab. I do not know whether to celebrate the lecture jump or panic about the practical.

What is actually happening

- Lecture exams climbing (48 → 51 → 65 → 68).
- Lab practicals flat (42 → 48 → 54).
- Foundation work started in Week 3 helped lecture, not lab.
- Studying mostly in the same modality (read + recall).

WHAT THIS CASE IS TEACHING

Most real midterms are mixed. The work is honest reading without celebration or despair. Apply the PROTECT / PIVOT / ADD framework. The method that worked for lecture is not the same method that will work for lab practical. Lab is a different modality and needs different practice (specimen-based, spatial, hands-on).

WEEK 4 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

E · Evaluate last week

Did you do what you said you would? Be specific, not performative.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

Your Plan (P) becomes your Decision Plan on the next page.

WEEK 4 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

- 1

- 2

- 3

One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

WEEK 4 ONLY · PROTECT / PIVOT / ADD

Label each of your three commitments above with one of these tags. P = protect what is working. V = pivot away from what is not. A = add something the midterm showed you needed.

WEEK 5 · PHASE: TREAT

Point recovery.

Recovery shows up in leading indicators first.

Week 5 comes after the midterm hinge. Your study sessions can already be sharper, your retention better, your anxiety lower, while your grade is still catching up. Week 5 is about learning to read the early signs. Leading indicators precede lagging indicators by weeks. Quitting because the grade has not moved is the most common Week 5 failure.

Your focus this week

Notice three leading indicators that have changed since Week 1. Write them down. Resist the urge to overhaul anything that is working. The temptation in Week 5 is to change the plan because the grade has not moved. The work is to trust the plan.

What you will do

- ☐ Read Riley's case.
- ☐ Complete your Week 5 audit. Pay extra attention to Subjective and Assessment.
- ☐ List three leading indicators that have changed since Week 1.
- ☐ Write three sustainment commitments.
- ☐

AVAILABLE ONLINE Point Recovery Inventory (online)

Catalogues the leading indicators hidden in your audits. Helpful for spotting recovery before the grade catches up.

drsrennie-stack.github.io/STAT/hub.html

WEEK 5 · CASE STUDY

Riley's case.

The intervention is working. Riley is about to quit anyway.

CASE SNAPSHOT

Course	Anatomy & Physiology 2
Week 1 grade	56%
Current grade	61%
Most recent exam	64% (up from 48%)
Final exam weight	30%
Weeks until final	3

Riley in their own words

I have done everything you said. I switched to active recall. I am sleeping more, studying less but better. And my grade went up five points. Five. I have a final worth 30% in three weeks. Even if I get a 90% on it, I might still fail. I do not know if I can keep doing this for three more weeks if it might not be enough.

What is actually happening

- Study sessions feel different: 45 minutes of focus vs. 15 in Week 1.
- Recall is happening without effort.
- Anxiety is lower: slept seven hours before midterm vs. three before previous exam.
- Study group sessions are productive.
- Sixteen-point jump on midterm vs. previous exam.
- Riley is reading only the course grade as the signal.

WHAT THIS CASE IS TEACHING

Riley is in active recovery. The intervention is working. The leading indicators are unambiguous. But Riley is reading only the lagging indicator and concluding the work is not paying off. If Riley quits now, they lose a recovery that has already begun. Make the leading indicators visible. To yourself first.

WEEK 5 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

E · Evaluate last week

Did you do what you said you would? Be specific, not performative.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

Your Plan (P) becomes your Decision Plan on the next page.

WEEK 5 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

1

2

3

One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

WEEK 6 · PHASE: TREAT

Consolidation & integration.

Revising your plan is not failure. Refusing to revise when data calls for it is.

Week 6 introduces differential diagnosis: when the plan stops producing results, that is information, not failure. Some students see real progress on their primary failure mode by Week 5. Some do not. The most likely explanation is not that you are working too little; it is that the original diagnosis was incomplete.

Your focus this week

For most students, Week 6 is consolidation. Sustain the plan, deepen the practice, trust the rhythm. For some, Week 6 is course correction. Re-read your Week 1 diagnostic. Compare it to what the last five weeks have shown. If they no longer fit, revise.

What you will do

- ☐ Read Sam's case.
- ☐ Re-read your Week 1 diagnostic results.
- ☐ Complete your Week 6 audit comparing to Week 1 baseline, not just last week.
- ☐ Either sustain or revise: write three commitments accordingly.
- ☐

WEEK 6 · CASE STUDY

Sam's case.

The method is better. The exam scores are not. Differential diagnosis required.

CASE SNAPSHOT

Course	Anatomy & Physiology 2
Week 1 grade	52%
Current grade	55%
Midterm score	58%
Week 1 diagnosis	Method failure
Hours studied / week	20 – 22

Sam in their own words

I am studying better than I have studied in my entire college career. I can recall things. I can explain things. I went into the midterm feeling actually prepared. And I got a 58%. My method is better. So why are my scores still terrible? I knew the answer to question 12 on the midterm. I just blanked. My mind went somewhere else and I could not get back. That happened five times.

What is actually happening

- Method has objectively improved over five weeks.
- Study sessions are sharper.
- Recall in low-stakes practice is strong.
- On exams: blanking, shallow breathing, hands shaking, time disappearing.
- Sam mentioned these symptoms in three of five audits.
- None appeared in the Week 1 diagnostic because Sam did not know to flag them.

WHAT THIS CASE IS TEACHING

The Week 1 diagnosis was right, but incomplete. Test anxiety has been masked by the obvious method failure. Method improvement alone cannot solve test anxiety. The next intervention requires different tools (exposure to exam-like conditions, breathing protocols, possible counseling support). Re-read your own Week 1 diagnostic. Look for what it missed. Revising your plan in Week 6 is not failure. It is information. Refusing to revise when the data is asking you to is failure. Sam is being given the chance to listen. So are you.

WEEK 6 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

E · Evaluate last week

Did you do what you said you would? Be specific, not performative.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

Your Plan (P) becomes your Decision Plan on the next page.

WEEK 6 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

1

2

3

One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

WEEK 7 · PHASE: MONITOR

Final exam runway.

Triage is the central skill of recovery. Not weakness.

Week 7 is the runway. The final is in two to three weeks. The work shifts from building habits to deploying them under exam pressure on cumulative material you cannot fully cover. Triage matters more than effort. You have been practicing triage for six weeks. Now apply it to the highest-stakes exam of the course.

Your focus this week

Produce a realistic prioritized plan you can actually execute over the next two weeks. The plan is the assignment. Map your final exam topic weights against your confidence map. Spend the most time where the highest weight meets the lowest confidence. Active recall, not passive review. Spaced practice, not crammed sessions.

What you will do

- ☐ Read Casey's case.
- ☐ Complete your Week 7 audit with a topic-by-topic confidence map.
- ☐ Use the Final Exam Triage tool on the case page.
- ☐ Write three exam-focused commitments and one 24-hour action.
- ☐

AVAILABLE ONLINE Final Exam Triage (online)

The triage table on paper here, automated online. Plug in topic weight, confidence, and time. Outputs your prioritized 14-day plan. Required this week.

drsrennie-stack.github.io/STAT/hub.html

WEEK 7 · CASE STUDY

Casey's case.

Fourteen days. Twelve chapters. Seven mastered. The math of triage.

CASE SNAPSHOT

Course	Anatomy & Physiology 2
Week 1 grade	51%
Current grade	60%
Midterm score	67% (up from 50%)
Final weight	30%
Days until final	14
Chapters mastered	7 of 12

Casey in their own words

I have actually done the work. But I look at the syllabus for the final and I see twelve chapters and I have basically not touched five of them. I have fourteen days. I cannot study five chapters from scratch in fourteen days while also reviewing the seven I do know. Part of me wants to spread my time evenly. Another part wants to triple down on what I know. Both feel wrong.

What is actually happening

- Method, foundation, and team work are paying off.
- Mastery achieved on 7 of 12 final-exam chapters.
- Five chapters un-studied include three of the four highest-weighted exam topics.
- Seven mastered chapters include three of the lowest-weighted topics.
- Casey has not yet looked at the topic weighting.

WHAT THIS CASE IS TEACHING

Casey's answer is sitting in the syllabus. The triage answer is not "study evenly" or "abandon the five." It is "study the highest-weighted topics first, regardless of which list they came from." The three highest-weighted topics are in the unstudied five. That makes them the highest priority. Triage is where will an hour of study produce the most points, not where will it feel best to invest.

WEEK 7 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

E · Evaluate last week

Did you do what you said you would? Be specific, not performative.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

Your Plan (P) becomes your Decision Plan on the next page.

WEEK 7 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

- 1

- 2

- 3

One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

WEEK 7 ONLY · FINAL EXAM TRIAGE

For each remaining exam topic, score (1) exam weight, (2) your current confidence, and (3) hours estimated to mastery. Spend time where (weight × low confidence) is highest. Use the table on the next page.

WEEK 7 · FINAL EXAM TRIAGE TOOL

Final exam triage table.

Fill this table before you start studying. The math is where the recovery is.

TOPIC / CHAPTER	EXAM WEIGHT	CONFIDENCE (1-5)	HOURS TO MASTER	PRIORITY

HOW TO USE THIS TABLE

(1) List every topic on the final, weighted accurately by your syllabus. (2) Score your confidence honestly. (3) Estimate hours required. (4) Rank priority: highest weight $\times$ lowest confidence first. Allocate your remaining study days to the top of the list.

WEEK 8 · PHASE: MONITOR

Definitive care & discharge.

What survives the program is the program.

Week 8 is the capstone. The work of recovery has been done, or it has not. This week is about looking honestly at what you have changed at the level of how you operate, and deciding what carries forward. In a hospital, the discharge plan is what determines whether the patient stays out of the hospital or returns within thirty days. The same is true here.

Your focus this week

Identify three categories of habits: those likely to survive without effort, those that will require deliberate maintenance, and those that will fade unless you replace the structure. Then build the replacement structure. The plan you make this week is for the next eight weeks without this workbook, not for the last exam itself.

What you will do

- ☐ Read Taylor's case.
- ☐ Complete your Week 8 audit, comparing to Week 1.
- ☐ Use the Discharge Planning worksheet in Part 4.
- ☐ Write three commitments for the eight weeks after this workbook ends.
- ☐

AVAILABLE ONLINE Discharge Planning (online)

The discharge worksheet, auto-structured. Pairs with the worksheet in Part 4. Use both.

drsrennie-stack.github.io/STAT/hub.html

WEEK 8 · CASE STUDY

Taylor's case.

The program worked. Discharge planning is what makes it stick.

CASE SNAPSHOT

Course	Anatomy & Physiology 2 (second attempt)
Week 1 grade	47%
Final score	72%
Course grade	71%
Week 1 diagnosis	Affective crisis (primary)
Next semester	A&P lab, anatomy, microbiology

Taylor in their own words

I passed. That feels strange to type. I have not passed an A&P course before. I do not know which parts I figured out for real and which parts were just the structure of being in this program every week. What happens when I am alone again.

What is actually happening

- Method changes likely to survive without effort.
- Weekly self-audit will require deliberate maintenance.
- Accountability structure will fade unless replaced.
- Shame, low self-efficacy, anticipatory anxiety did not resolve. Just stabilized.
- Stabilized, not transformed.

WHAT THIS CASE IS TEACHING

A weak discharge plan ("I will keep doing what I have been doing") produces relapse. A strong discharge plan names which structures to replace, which habits to keep, and which signals would call you back to recovery work. Make a strong one. The work of Week 8 outlasts the program. And remember: Taylor stabilized, not transformed. Stabilization is enough. Even if your outcome is worse than Taylor's, that is still recoverable. A failed class is a delay, not an ending. The only outcome that does not recover is the one where you decide you are the failure. Do not decide that. The data is not in yet. The data is never in yet.

WEEK 8 · C · CURRENT STATE AUDIT

Audit.

About 20 minutes. The SOAP structure: Subjective, Objective, Assessment, Plan.

E · Evaluate last week

Did you do what you said you would? Be specific, not performative.

S · Subjective

How did this week feel? What is your internal weather?

O · Objective

The data. Hours studied. Topics covered. Assessment scores. Office hours visits.

A · Assessment

What pattern do you see? What is the data telling you?

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WEEK 8 · D · DECIDE

Decision plan.

Three specific commitments. One 24-hour action. One prediction.

Three commitments for this week

1

2

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One action in the next 24 hours

Concrete, small, immediate. Something you will do today or tomorrow.

Your prediction

How do you expect this week to go? What might get in the way?

PART 4

After STAT.

When your grade can't wait.

PART 4 · WHAT SURVIVES THE PROGRAM

Discharge planning.

20 – 30 minutes. The last and most important worksheet.

In a hospital, the discharge plan determines whether the patient stays out of the hospital or returns within thirty days. The same is true here. Without a discharge plan, the structure of this workbook stops, and so do the habits it was holding up. Build the structure that replaces this one.

A.**Likely to survive without effort**

Habits that now feel like the default. Going back to the old way would feel wrong. List what is true for you.

B.**Will require deliberate maintenance**

Habits that work, but only because you have been doing them on a schedule. Will fade if you do not protect them.

C.**Will fade unless you replace the structure**

Habits that depended entirely on something external (this workbook, an accountability partner, a deadline). Name the replacement now.

IF YOU DID THE WEEK 1 ACCOUNTABILITY PARTNER

This is the moment to formalize it. A 30-minute weekly check-in that survived eight weeks is the strongest replacement structure you can build for yourself. The Continuing Study Group tool on the hub gives you a one-page template for turning your partner into a lasting habit. Use it now, while the muscle memory is fresh.

PART 4 · THE HAND-OFF

Recovery is not prevention.

The skills you used to recover are not all the skills you should use to stay well.

How you approach a course when you are failing is very different from how you approach a course you are starting fresh or doing well in. Recovery and prevention are different modes. They use different tactics, allocate different time, and measure different things. A student who keeps studying in recovery mode after the crisis is over will burn out. A student who tries to use prevention tactics during a crisis will fail. Knowing which mode you are in is half the work.

RECOVERY MODE	PREVENTION MODE
WHEN YOU USE IT	
You are below your goal grade, time is short, or your acuity is Urgent or Critical.	You are at or above your goal, time is normal, or your acuity is Stable.
COVERAGE STRATEGY	
Triage. Study the highest-weight, lowest-confidence topics. Accept gaps in low-yield areas.	Comprehensive. Cover all assigned material at adequate depth. Reach for connections.
TIME ALLOCATION	
Aggressive on a few topics. Stop spending time on anything that is not the bleed point.	Distributed across the syllabus. Time spent on enrichment, not just survival.
METHOD	
Active recall, retrieval practice, fewest possible tools. Cut everything that does not retain.	Active recall AND broader tools: discussions, application, reading for synthesis.
WHAT YOU MEASURE	
Did your grade move? Are you stable? Are leading indicators present?	Are you mastering the material? Can you teach it? Are you connecting across units?
WHAT SUCCESS LOOKS LIKE	
You pass. You stabilize. You are present for the next class. You did not give up.	You finish strong. You retain material into the next course. You build on it.

THE TRANSITION

When you finish this workbook, you are not in recovery anymore. You are between modes. Your discharge plan (previous page) bridges the two. Pick which prevention tactics you will add back in. Keep the recovery tactics that proved useful even outside crisis.

PART 4 · WHEN TO COME BACK

Warning signs.

Name three signals that would tell you to re-open this workbook.

Most academic crises do not arrive overnight. They build for two to four weeks before they show up on a transcript. The skill of monitoring (Phase IV) is reading the early signals before the crisis is in the gradebook. List the three signals that would tell future-you that the wheels are coming off, and what you will do when you see them.

1 WARNING SIGN #1

When I see this, I will:

2 WARNING SIGN #2

When I see this, I will:

3 WARNING SIGN #3

When I see this, I will:

YOU ARE ALLOWED TO COME BACK.

Returning to a recovery program is not failure. It is a working monitoring system. The students who relapse worst are the ones who refuse to re-enter recovery because they treat the first recovery as a one-time event. Recovery is a skill. You will use it more than once.

TEAR-OUT · THE WHOLE METHOD ON ONE PAGE

Quick reference.

C – O – D – E (every week)

- C** Current state audit. SOAP-note your week.
- O** Observe a case. Borrow what fits.
- D** Decide. Three commitments + one 24-hour action.
- E** Evaluate. Did you do what you said? Be honest.

Four phases

Triage	W1	Diagnose acuity. Identify bleeding points. Rank.
Stabilize	W2–3	Stop the worst method. Repair the most blocking gap.
Treat	W4–6	Read mixed data honestly. Sustain. Revise if needed.
Monitor	W7–8	Triage the final. Build a discharge structure.

Three acuities

Stable	B-range, > 4 weeks, broad confidence
Urgent	C-range, 2–4 weeks, specific gaps
Critical	≤ passing, < 2 weeks, broad uncertainty

Four failure modes

Time	Scarcity. The hours are not there.
Method	Passive review. Hours without retention.
Content	Foundation gaps blocking current material.
Affect	Anxiety, shame, avoidance, low self-efficacy.

MEDMASTERS COLLABORATIVE

When your grade can't wait.

Failure is what happened.

It is not who you are.

*Bring the workbook. Bring your honesty. Bring your refusal to fail
quietly. The rest is method.*



COMPANION TOOL HUB

drsrennie-stack.github.io/STAT/hub.html

Scan for every interactive tool in this workbook.

DR. SHARILYN RENNIE

medmasterscollaborative.com